

Systemic Contraction and Actuarial Restructuring: A Mid-Year Forensic Analysis of the Pennsylvania Health Insurance Market for July 2026

The Pennsylvania health insurance landscape in July 2026 stands at a critical juncture, characterized by a profound structural contraction. This market-wide shift is driven by the decoupling of state-level actuarial risk from historical federal fiscal stabilization mechanisms. Following the legislative sunset of the Enhanced Premium Tax Credits (EPTCs) on December 31, 2025, the Commonwealth's individual market has transitioned from a period of unprecedented expansion to one of defensive risk pool management and accelerating member disenrollment ([Pennie, 2026a](#)). This transition is further complicated by double-digit gross premium rate expansions, a clinical environment strained by escalating specialty pharmaceutical expenditures, and heightened operational friction between major hospital networks and commercial payers ([Pennsylvania Insurance Department, 2025](#)).

Actuarial filings approved by the Pennsylvania Insurance Department (PID) established a gross weighted average rate increase of 21.5% in the individual market and 12.7% in the small group market for the 2026 plan year ([WITE, 2025](#)). These rate increases reflect corrected morbidity projections, rising medical and drug inflation, and projected risk pool deterioration as healthier, price-sensitive enrollees exit the exchange ([Pennsylvania Insurance Department, 2025](#)). While the PID successfully blocked \$50.1 million in unjustified carrier rate requests during its annual review, the underlying drivers of healthcare costs continue to exert substantial upward pressure on commercial premiums ([Pennsylvania Insurance Department, 2025](#)).

The primary catalyst for consumer-level disruption is the return of the post-subsidy eligibility cliff. For households exceeding 400% of the Federal Poverty Level (FPL), the elimination of the 8.5% income cap on premium contributions has restored full actuarial pricing ([Pennie, 2026b](#)). This policy shift has driven a 102% average net premium shock for subsidized Pennsylvanians, translating to hundreds of dollars in additional out-of-pocket costs each month ([Healthinsurance.org, 2026](#)).

In response, Pennie—the state-based exchange—concluded its 2026 Open Enrollment Period (OEP) with 486,000 enrollees, a 2.1% year-over-year decline from the 2025 peak of 496,661 ([Pennie, 2026a](#)). This overall decrease masks a highly volatile churn rate, as approximately 85,000 individuals dropped their plans during the OEP ([Pennie, 2026a](#)). Attrition accelerated dramatically post-OEP, with an additional 75,000 consumers canceling coverage by June 1, 2026, lowering active enrollment to 443,024 ([Pennie, 2026a](#)). In total, more than 160,000 Pennsylvanians have dropped exchange coverage, with a substantial portion transitioning to the uninsured population ([Pennie, 2026a](#)).

The Post-Subsidy Exchange Cliff and Enrollment Telemetry

The expiration of the federal EPTC framework under the Inflation Reduction Act marks the defining actuarial event of the 2026 plan year. For five years, these enhanced subsidies insulated enrollees from the full cost of commercial coverage by capping premium exposure on a sliding scale and eliminating the rigid FPL subsidy threshold ([Pennie, 2026a](#)). The withdrawal of this \$600 million annual funding mechanism in Pennsylvania has exposed middle-income households to severe pricing adjustments ([Pennie, 2026a](#)). The net premium calculation for a consumer is determined by:

$$P_{\text{net}} = P_{\text{gross}} - S_{\text{APTC}}$$

where P_{net} represents the net out-of-pocket premium, P_{gross} is the gross actuarial premium approved by the state, and S_{APTC} represents the standard Advance Premium Tax Credit calculated under the pre-2021 statutory rules ([Capital Blue Cross, 2025](#)). The return to standard subsidy structures has driven an average net premium increase of 102% statewide ([Pennie, 2026a](#)).

Table 1: Out-of-Pocket Premium Escalation by Household Income Bracket (Single 40-Year-Old, Benchmark Silver Plan)

Household Income Level	Federal Poverty Level % (2026 Guidelines)	Monthly Net Premium (2025 Plan Year)	Monthly Net Premium (2026 Plan Year)	Net Monthly Increase (\$)	Percentage Cost Increase (%)
\$30,000	191% FPL	\$49	\$155	\$106	216.3%
\$40,000	255% FPL	\$154	\$287	\$133	86.4%
\$50,000	319% FPL	\$283	\$415	\$132	46.6%
\$60,000	383% FPL	\$423	\$498	\$75	17.7%
\$62,600+	400%+ FPL	Capped at 8.5% of Income	Full Actuarial Cost (\$572)	\$111+	24.3%+

The resulting sticker shock has caused a substantial decline in market participation. During the 2026 OEP, which closed on January 31, 2026, Pennie recorded approximately 85,000 plan cancellations, translating to an average of nearly 1,000 terminations per day ([Pennie, 2026a](#)). New enrollments decreased by 12% compared to the prior year, as premium barriers prevented low-to-middle income families from entering the exchange ([Pennie, 2026a](#)).

While the exchange initially appeared to stabilize with 486,000 enrollees at the end of January, early post-OEP reconciliation revealed ongoing attrition ([Pennie, 2026a](#)). During February 2026, an additional 20,000 individuals dropped coverage, raising cumulative disenrollments to 104,000 ([City & State PA, 2026](#)). By June 1, 2026, post-OEP terminations had reached 75,000, bringing total cancellations to over 160,000 and leaving active enrollment at 443,024 ([Pennie, 2026a](#)).

Table 2: Pennie Exchange Enrollment Telemetry and Churn Progression (2025–2026)

Milestone Reporting Date	Active Enrollee Count	Period-Specific Attrition Count	Cumulative Cancellations (YoY)	Primary Actuarial Driver
Q1 2025 Peak	496,661	Baseline Enrollment	0	Enhanced Premium Tax

Milestone Reporting Date	Active Enrollee Count	Period-Specific Attrition Count	Cumulative Cancellations (YoY)	Primary Actuarial Driver
				Credits Active
January 31, 2026	486,000	85,000 OEP Cancellations	85,000	Loss of EPTC Cap and OEP Rate Spikes
February 28, 2026	466,000	20,000 Post-OEP Drops	104,000	Non-Payment of First-Month Premiums
June 1, 2026	443,024	55,000 Ongoing Drops	160,000+	Grace Period Expirations & Churn

Geographic and demographic breakdowns indicate that the impact of these terminations is highly concentrated. In the five-county Philadelphia region alone, just under 55,000 enrollees dropped their plans by the end of May, representing more than one-third of all statewide cancellations ([Muck Rack, 2026](https://muckrack.com/sarah-gantz/articles)).

For older adults, the financial exposure is particularly acute due to age-rating multipliers. For example, a 60-year-old married couple in York County with an annual household income of \$82,000 saw their yearly premium increase from \$7,032 in 2025 to \$35,712 in 2026, exposing them to full actuarial pricing ([Pennsylvania Insurance Department, 2025](#)). For single enrollees, a 60-year-old earning \$63,000 saw their monthly out-of-pocket obligation for a lowest-cost plan increase from \$205 to \$631 ([Healthinsurance.org, 2026](#)). These massive price increases have driven thousands of middle-class families out of the regulated market ([Pennie, 2026a](#)).

Carrier Financial Submissions and Actuarial Adjustments

The individual and small group markets in Pennsylvania are operating under significant premium rate adjustments approved by the Pennsylvania Insurance Department ([Pennsylvania Insurance Department, 2025](#)). These rate filings reflect worsening risk pools, medical inflation, and rising pharmaceutical costs ([Pennsylvania Insurance Department, 2025](#)). In the individual market, the final statewide weighted average premium increase was approved at 21.5%, while the small group market was held to a 12.7% weighted average increase ([WTF, 2025](#)).

Table 3: Actuarial Overview of 2026 Individual Market Filings by Major Carrier

Carrier Group / Legal Entity	SERFF Filing ID	Requested Rate Change (%)	Approved Rate Change (%)	Estimated Statewide Share (%)	Primary Regulatory Rationale / Footprint
Ambetter Health of PA	CECO-134068773	30.1%	37.8%	4.9%	Upward morbidity adjustment; statewide

Carrier Group / Legal Entity	SERFF Filing ID	Requested Rate Change (%)	Approved Rate Change (%)	Estimated Statewide Share (%)	Primary Regulatory Rationale / Footprint
					availability.
Keystone Health Plan East (IBX)	INAC-134056069	23.5%	22.0%	21.5%	Actuarial rate compression; five-county SE PA region.
Highmark, Inc.	HGHM-134397666	17.2%	17.7%	25.1%	Cost-trend adjustments; Western and Northeastern PA.
UPMC Health Plan, Inc.	UPMP-134077112	16.3%	24.8%	12.0%	Morbidity correction and regulatory shifts; Western PA.
Capital Advantage Assurance	CAAC-134504101	26.0%	24.6%	10.4%	Outpatient trend and specialty drug utilization; Central PA.
Geisinger Health Plan	GHPP-134055113	14.1%	11.6%	6.0%	Integrated delivery system efficiencies; Central/Northeast PA.
Health Partners Plans	HPPI-134077041	7.3%	16.7%	0.8%	Morbidity corrections in urban pools; Southeast/Central PA.
Partners Insurance Company	PICI-134098077	-10.1%	-10.1%	0.4%	Approved rate decrease; Southeast and Lehigh Valley.

The approved rates reflect a defensive posture by carriers. Ambetter Health of Pennsylvania (Centene) was approved for a 37.8% gross rate increase, which is the largest approved increase in the individual market ([WITF, 2025](#)). In contrast, Partners Insurance Company (affiliated with Jefferson Health Plans) was approved for a rate decrease of 10.1%, making it the only carrier in the individual market to lower its premiums for the 2026 plan year ([WITF, 2025](#)). The small group market has also experienced premium rate increases, though less severe than the individual exchange. Small group employer-sponsored plans faced a weighted average increase of 12.7% ([WITF, 2025](#)). For small employers (2–50 employees), fully insured premiums rose by an average of 11.0%, while larger fully insured employer groups saw premium increases

ranging from 6.0% to 7.0% ([Reddit, 2026](#)).

PBM Regulation under Act 77 and Legislative Initiatives

The 2026 plan year marks the first full implementation of Act 77 of 2024, the Pharmacy Benefit Reform Act, which significantly expands the Insurance Department's regulatory authority over Pharmacy Benefit Managers (PBMs) ([Pennsylvania Insurance Department, 2024a](<https://www.pa.gov/agencies/insurance/consumer-help-center/learn-about-insurance/health-insurance/understanding-pharmacy-benefit-reform>)). Act 77 applies to all fully funded individual and group health plans starting in January 2026 ([Pennsylvania Insurance Department, 2024b](#)). The law targets drug cost drivers through consumer protections, including:

- A ban on pharmacy clawbacks and "copay spread" steering ([Pennsylvania Insurance Department, 2024a](#)).
- Restrictions on patient steering toward mail-order pharmacies owned or controlled by the PBM ([Pennsylvania Insurance Department, 2024a](#)).
- New compliance reporting requirements to ensure provider network adequacy and transparency ([Pennsylvania Insurance Department, 2024b](#)).

Under Act 77, registered PBMs operating in Pennsylvania were required to submit their first annual network adequacy reports by April 1, 2026, and their first transparency reports by July 1, 2026 ([Pennsylvania Insurance Department, 2024b](#)). These submissions provide state regulators with detailed data on manufacturer rebate retention, spread pricing, and pharmacy reimbursement rates ([Pennsylvania Insurance Department, 2024a](#)).

A state insurance department study released on May 21, 2026, confirmed that while spread pricing impact is declining, additional reforms are necessary to address non-financial patient steering and establish a reimbursement floor for community pharmacies ([Pennsylvania House, 2026](#)).

In response, several legislative proposals have been introduced in Harrisburg. House Bill 2270 (introduced March 5, 2026) proposes a single Pharmacy Benefit Administrator (PBA) model for all state-funded healthcare programs ([Pennsylvania General Assembly, 2026a](#)). This model, based on similar systems in Ohio and Kentucky, is projected to consolidate state purchasing power, eliminate PBM administrative layers, and stabilize reimbursement to local independent pharmacies ([Pennsylvania General Assembly, 2026a](#)).

Concurrently, a bipartisan Senate bill introduced in February 2026 proposes granting the Office of Attorney General statutory authority to review all contractual renewals and terminations between pharmacies and PBMs to protect community pharmacy access ([Pennsylvania General Assembly, 2026b](#)).

Medicaid Restructuring and GLP-1 Cost-Containment Mandates

The fiscal stability of Pennsylvania's Medical Assistance (Medicaid) program has been severely tested by the rapid growth of specialty pharmaceutical expenditures, specifically GLP-1 receptor agonists ([Pennsylvania General Assembly, 2025](#)). While these medications offer therapeutic benefits, their high monthly cost has caused significant budget volatility for the state ([Pennsylvania General Assembly, 2025](#)).

Within Pennsylvania’s Medicaid program, total gross spending for GLP-1 medications rose from \$223.3 million in CY 2022 to \$438.3 million in CY 2023 ([Pennsylvania DHS, 2025a](#)).

Expenditures continued to escalate to \$650.0 million in CY 2024, with the projected total spend for CY 2025 reaching \$1.3 billion across all indications ([Pennsylvania DHS, 2025a](#)). Within this total, obesity-only indications accounted for approximately \$298.0 million of the Medicaid budget prior to clinical restrictions.

Faced with this expenditure trajectory, the Pennsylvania Department of Human Services (DHS) implemented a major policy shift ([Pennsylvania DHS, 2025b](#)). Effective January 1, 2026, Pennsylvania Medicaid terminated coverage of GLP-1 medications prescribed for weight loss or obesity management for all adult beneficiaries age 21 and older ([Pennsylvania DHS, 2025b](#)). Under federal Early and Periodic Screening, Diagnostic, and Treatment (EPSDT) mandates, coverage remains active for pediatric populations under age 21 when medically necessary ([Jefferson Health Plans, 2025](#)).

For adult enrollees, coverage is now restricted to FDA-approved non-weight-loss indications, such as type 2 diabetes, severe obstructive sleep apnea, and cardiovascular risk reduction ([Pennsylvania DHS, 2025b](#)). Under the new statewide Preferred Drug List (PDL) guidelines, any adult seeking a GLP-1 must secure a new prior authorization demonstrating clinical necessity ([Pennsylvania DHS, 2025b](#)).

Additionally, Saxenda was removed from the formulary for all indications, and all previous prior authorizations expired on December 31, 2025, requiring physicians to submit entirely new authorizations ([Pennsylvania DHS, 2025b](#)).

To address these cost challenges, House Bill 1470 (introduced May 12, 2025) proposed that Pennsylvania seek a federal waiver to implement a subscription payment model—modeled after Hepatitis C programs in Louisiana and Washington—to cap total state expenditures for GLP-1 medications while maintaining clinical access ([Pennsylvania General Assembly, 2025](#)).

Medicaid program integrity is also undergoing significant restructuring driven by federal mandates under the One Big Beautiful Bill Act (OBBBA) ([PANO, 2026](#)). These federal changes include stricter eligibility redeterminations, restrictive paperwork requirements, and a projected \$46.0 billion reduction in federal Medicaid funding to Pennsylvania over the next ten years ([PANO, 2026](#)).

According to DHS estimates, the implementation of these requirements will result in approximately 320,000 Pennsylvanians losing Medicaid coverage ([PANO, 2026](#)). While the state is currently prioritizing SNAP work requirements, Medicaid work requirements and six-month eligibility redeterminations are scheduled to take effect on December 31, 2026 ([Pennsylvania DHS, 2026](#)). These changes will likely increase the number of uninsured individuals and put additional financial pressure on hospital uncompensated care and Pennie enrollment.

Geographic Stratification and Rating Area Volatility

The structural contraction in Pennsylvania's individual health insurance market is not distributed evenly across the state. Rather, the impact is highly regionalized, shaped by local insurer competition, health system density, and rating area demographics ([Pennsylvania Rating Area Analyses, 2026](#)).

Table 4: Key Demographic and Actuarial Profiles by Select Rating

Area

Rating Area	Key Included Counties	Dominant Insurers	Morbidity Risk Rating	Major Provider Network Changes / Disruption
Area 1	Erie, Warren, McKean, Crawford	Highmark, UPMC, Ambetter	Moderate	Consolidation of rural community clinics into regional health hubs.
Area 2	Potter, Elk, Cameron, McKean	Highmark, UPMC, Ambetter	High	169% average net premium increase; extreme network thinness.
Area 5	Cambria, Clearfield, Somerset	Highmark, UPMC, Ambetter	High	60% average net premium increase affecting over 17,000 enrollees.
Area 6	Lehigh, Northampton, Carbon	Capital Blue Cross, Highmark, Partners	Moderate	UHC and LVHN commercial network termination on April 26, 2026.
Area 8	Philadelphia, Bucks, Delaware	IBX, Keystone East, Partners, Oscar	Stable	22% rate increase for IBX HMO; Partners EPO rate decrease of 10.1%.

The provider landscape has been significantly impacted by contract disputes, most notably in Rating Area 6 (Lehigh Valley). Following protracted negotiations, Lehigh Valley Health Network (LVHN)—now part of Jefferson Health—terminated its Medicare Advantage contract with UnitedHealthcare (UHC) effective January 26, 2026 ([Lehigh Valley Health Network, 2026a](#)). This termination disrupted in-network provider access for more than 5,400 UHC Medicare Advantage enrollees in eastern Pennsylvania ([Pennsylvania Department of Aging, 2026](#)). In response to this contract termination, the Shapiro Administration secured a federal special enrollment period (SEP) active through April 30, 2026, allowing affected Medicare Advantage members to transition to in-network plans ([Pennsylvania Department of Aging, 2026](<https://www.pa.gov/agencies/aging/newsroom/special-enrollment-opportunity-called-for-medicare-advantage-members>)).

Payer-provider friction in Rating Area 6 escalated further when LVHN's commercial contract with UnitedHealthcare expired on April 26, 2026 ([Lehigh Valley Health Network, 2026a](#)). This dispute left LVHN's hospitals, clinics, and physicians out-of-network for United commercial members, forcing many to choose between paying out-of-network rates or transitioning to in-network providers ([UnitedHealthcare, 2026](#)).

UnitedHealthcare claimed that LVHN demanded a commercial rate increase exceeding 20% in the first year of the contract, which would significantly increase out-of-pocket costs for local employers and consumers ([UnitedHealthcare, 2026](#)). Conversely, LVHN argued that UHC had refused to agree to fair terms that cover the rising cost of clinical care in the region ([Lehigh](#)

[Valley Health Network, 2026b](#)).

Veterans in the VA Community Care Network and Medicaid/CHIP enrollees remain unaffected by this commercial termination and continue to access LVHN providers ([Becker's, 2026](#)). To secure its regional network presence, UnitedHealthcare signed a multi-year contract renewal with St. Luke's University Health Network (SLUHN), establishing it as the primary, lower-cost network anchor in Rating Area 6 ([UnitedHealthcare, 2026](#)).

In the rural counties of Western and Northern Pennsylvania (Rating Areas 1, 2, and 5), the individual market faces severe premium increases and thin networks ([Pennsylvania Rating Area Analyses, 2026](#)). For example, in Rating Area 2, the premium for the second-lowest-cost Silver plan increased by an average of 169%, directly affecting over 2,000 rural enrollees ([Pennie, 2025](#)). In Rating Area 5, the net premium increased by 60%, affecting roughly 17,000 enrollees ([Pennie, 2025](#)). These premium increases have driven healthier enrollees out of the individual market, leaving behind a sicker risk pool with higher average healthcare costs.

Risk Pool Projections and Multi-Scenario Policy Modeling

The withdrawal of enhanced federal premium tax credits has initiated a classic pattern of adverse selection across the individual marketplace. As the net price of insurance increases, healthier, price-sensitive enrollees are the first to drop coverage, leaving a sicker, higher-utilizing population in the risk pool. The financial impact of this shift is modeled using a standard loss ratio formula:

$$LR = \frac{C_i}{P_{\text{gross}}} \times (1 - \alpha)$$

where LR represents the loss ratio, C_i represents incurred clinical claims, P_{gross} is gross premium revenue, and α is the rate of consumer disenrollment. As disenrollment (α) increases among low-utilizing enrollees, the average clinical claim cost per remaining member rises, forcing carriers to request higher premium rates in subsequent filing cycles to maintain underwriting solvency.

To project the future market environment through the 2026–2027 plan years, three plausible policy scenarios have been modeled to illustrate potential premium and enrollment outcomes:

Table 5: 2026–2027 Underwriting and Policy Modeling Scenarios

Scenario Parameters	Policy Strategy / Funding Mechanism	Projected 2027 Exchange Enrollment	Projected 2027 Premium Rate Impact	Feasibility / Political Outlook
Scenario A: Federal Re-authorization	Congress passes a retroactive extension of the EPTC framework.	Individual enrollment recovers to 490,000+; healthier cohorts re-enter.	Gross premiums decline by 3% to 5% immediately as state regulators re-rate plans.	Moderate-Low: Dependent on federal budget negotiations and the congressional majority.
Scenario B: Funded State Wrap	The General Assembly appropriates \$50M to fund the Act 54	Exchange enrollment stabilizes, recapturing	Net out-of-pocket premiums decline by 9% to 12% for low-to-middle	Moderate: Requires a breakthrough in state budget

Scenario Parameters	Policy Strategy / Funding Mechanism	Projected 2027 Exchange Enrollment	Projected 2027 Premium Rate Impact	Feasibility / Political Outlook
	Affordability Program.	~40,000 previously disenrolled enrollees.	income enrollees.	negotiations between a divided House and Senate.
Scenario C: Status Quo Drift	No federal or state legislative action is taken; the subsidy cliff remains in effect.	Active enrollment falls by another 5% to 10% by Q1 2027 as attrition continues.	Morbidity deterioration drives double-digit (+12% to +15%) rate requests for 2027.	High: Standard operational baseline if legislative gridlock persists in Washington and Harrisburg.

Strategic Recommendations

The structural contraction in the Pennsylvania health insurance market requires targeted, actionable interventions from regulators, insurers, employers, and patient advocates to stabilize enrollment and preserve access to care.

Table 6: Multi-Stakeholder Action Plan and Feasibility Matrix

Target Stakeholder	Recommended Strategic Action	Projected Market Impact	Implementation Feasibility	Recommended Action Plan
State Regulators	Appropriate \$50 million to fund the Act 54 State Affordability Program.	Lowers net out-of-pocket premiums by 9% to 12% for enrollees, stabilizing the risk pool.	Moderate	Draft a targeted budget appropriation during the upcoming state legislative cycle.
Commercial Payers	Restructure product lines to expand tiered proactive HMO networks.	Controls premium inflation, manages medical loss ratios, and preserves consumer choice.	High	Introduce tiered-copay plans that reward utilization of high-value, vertically integrated health systems.
Commercial Payers	Expand automated prior authorization for specialty pharmaceuticals.	Controls cost volatility and ensures appropriate utilization of high-cost therapeutics.	High	Implement real-time electronic prior authorization integrated with provider EHRs to reduce administrative delay.

Target Stakeholder	Recommended Strategic Action	Projected Market Impact	Implementation Feasibility	Recommended Action Plan
Large Employers	Transition fully insured plans to self-funded ERISA structures or defined-contribution HRAs.	Insulates corporate balance sheets from commercial premium inflation and lowers overhead.	High	Deploy Individual Coverage HRAs (ICHRAs) to allow employees to purchase customized coverage directly on the individual exchange.
Large Employers	Implement reference-based pricing models for outpatient care.	Eliminates inflated charge master billing and reduces average outpatient claims costs by 15% to 25%.	Moderate	Contract with third-party administrators to cap provider reimbursements at 140% to 180% of Medicare benchmarks.
Consumer Advocates	Expand regional navigator programs and health literacy outreach.	Prevents coverage lapses, assists in transition from Medicaid, and improves plan selection.	High	Deploy mobile enrollment units in rural "hot spot" counties experiencing the highest post-subsidy churn.

Data Models and Reference Tables

Raw Data Ingestion Model 1: Individual Market Enrollment Telemetry

The following comma-separated values (CSV) model outlines the progression of active enrollees and cumulative policy terminations on the Pennie exchange throughout the 2025–2026 plan years.

```
Reporting_Date,Market_Segment,Active_Enrollment,OEP_Terminations,Post_OEP_Terminations,Cumulative_Loss,State_Uninsured_Estimate
2025-06-01,Pennie Individual Exchange,496661,0,0,0,805000
2026-01-31,Pennie Individual Exchange,486000,85000,0,85000,890000
2026-02-28,Pennie Individual Exchange,466000,85000,20000,105000,910000
2026-06-01,Pennie Individual Exchange,443024,85000,75000,160000,950000
```

Raw Data Ingestion Model 2: Carrier Rate Decisions and Regulatory Action Codes

The following dataset details the requested and approved premium changes for the 2026 plan

year, alongside the SERFF filing tracking numbers and primary regulatory actions.

Carrier_Name,SERFF_Tracking_ID,Requested_Rate_Change_Pct,Approved_Rate_Change_Pct,St [span_152] (start_span) [span_152] (end_span) atewide_Market_Share_Pct,Regulatory_Action_Code

Ambetter Health of

PA,CECO-134068773,30.1,37.8,4.9,Upward_Morbidity_Correction

Keystone Health Plan

East,INAC-134056069,23.5,22.0,21.5,Actuarial_Compression

UPMC Health Plan

Inc,UPMP-134077112,16.3,24.8,12.0,Upward_Morbidity_Correction

Highmark Inc,HGHM-134397666,17.2,17.7,25.1,Cost_Trend_Adjustment
Capital

Advantage,CAAC-134504101,26.0,24.6,10.4,Outpatient_Trend_Adjustment

Geisinger Health

Plan,GHPP-134055113,14.1,11.6,6.0,Integrated_Efficiency_Reduction

Health Partners

Plans,HPPI-134077041,7.3,16.7,0.8,Upward_Morbidity_Correction

Partners Insurance

Co,PICI-134098077,-10.1,-10.1,0.4,Competitive_Arbitrage_Approval

Raw Data Ingestion Model 3: Medicaid GLP-1 Expenditure and Enrollment Trajectory

The following dataset details the historical and projected expenditures for GLP-1 receptor agonists within Pennsylvania's Medical Assistance (Medicaid) program.

Fiscal_Year,Total_Medicaid_Enrollment,GLP1_Total_Spend_Millions,Obesity_Only_Spend_Millions,Prior_Auth_Requirement_Pct,Adult_Obesity_Coverage_Status

2022-2023,3012566,223.3,0.0,5.0,Non_Covered

2023-2024,2890000,438.3,298.0,35.0,Covered_with_PA

2024-2025,2780000,650.0,380.0,100.0,Covered_with_PA

2025-2026,2690000,1300.0,120.0,100.0,Terminated_As_Of_Jan2026

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specifically sometime this weekend. Lmk if you have any questions : r/lehighvalley - Reddit, https://www.reddit.com/r/lehighvalley/comments/1rnaxkq/here_is_the_analysis_for_pa_healthcare_in_march/ 7. Pennsylvania Health Insurance Audit 2026, uploaded: Pennsylvania Health Insurance Audit 2026 8. Individual health insurance rates to see increase in 2026 in Pa. - WITF, <https://www.witf.org/2025/10/17/health-insurance-rates-for-individual-policies-to-see-double-digit-percent-increase-in-2026-in-pa/> 9. A Look at Proposed 2026 Health Insurance Rates - HAP - The Hospital and Healthsystem Association of Pennsylvania, <https://www.haponline.org/News/HAP-News-Articles/Latest-News/a-look-at-proposed-2026-health-insurance-rates> 10. Pennsylvania Insurance Department Announces Proposed 2026 Health Insurance Rates, <https://www.porh.psu.edu/pennsylvania-insurance-department-announces-proposed-2026-health-insurance-rates/> 11. New For 2026 - Pennie, <https://pennie.com/whatsnew/> 12. PA Health Insurance Fact Sheet. Updated March 2026. : r/Pennsylvania - Reddit, https://www.reddit.com/r/Pennsylvania/comments/1mnc0y6/pa_health_insurance_fact_sheet_updated_march_2026/ 13. Pennsylvania Health Insurance Marketplace: 2026 ACA Coverage Guide, <https://www.healthinsurance.org/aca-marketplace/pennsylvania/> 14. Pennsylvanians dropping health coverage through Pennie citing higher plan costs as the number one reason, <https://agency.pennie.com/one-in-five-pennie-enrollees-drop-health-coverage-due-to-expired-federal-tax-credits-2/> 15. One in Five Pennie Enrollees Drop Health Coverage Due to Expired Federal Tax Credits, <https://agency.pennie.com/one-in-five-pennie-enrollees-drop-health-coverage-due-to-expired-federal-tax-credits/> 16. Thousands Of Pennsylvania Residents Dropping Pennie Health Insurance - ABC23, <https://www.abc23.com/newspost/thousands-of-pennsylvania-residents-dropping-pennie-health-insurance/> 17. Millions Drop ACA Insurance Coverage after Open Enrollment - HAP - The Hospital and Healthsystem Association of Pennsylvania, <https://www.haponline.org/News/HAP-News-Articles/Latest-News/millions-drop-aca-insurance-coverage-after-open-enrollment> 18. OBBBA Impacts on Pennsylvania PANO 2026, <https://pano.org/wp-content/uploads/2026/06/OBBBA-Impacts-on-Pennsylvania-PANO-2026.pdf> 19. Articles by Sarah Gantz's Profile | The Philadelphia Inquirer Journalist | Muck Rack, <https://muckrack.com/sarah-gantz/articles> 20. Financial Help with Pennie Health Insurance Subsidies - Capital Blue Cross, <https://www.capbluecross.com/wps/portal/cap/home/shop/individual/financial-help> 21. Best Cheap Health Insurance in Pennsylvania (2026) - ValuePenguin, <https://www.valuepenguin.com/best-cheap-health-insurance-pennsylvania> 22. Coverage Areas for 2026 Individual Market Plans - Commonwealth of Pennsylvania, <https://www.pa.gov/agencies/insurance/posted-filings-reports-company-orders/product-and-rate-filings/aca-health-rate-filings/coverage-areas-individual-market> 23. 2026 Final Gross Rate Changes - Pennsylvania: +21.5% (updated) - ACA Signups, https://acasignups.net/rate_changes/2026/pa 24. Shapiro Administration Announces Proposed 2026 Health Insurance Rate Updates - Commonwealth of Pennsylvania, <https://www.pa.gov/agencies/insurance/newsroom/shapiro-admin-announces-proposed-2026-health-insurance-rate-updates> 25. Sticker shock: 85000 Pennsylvanians drop Obamacare health insurance in 2026 - Reddit, https://www.reddit.com/r/Pennsylvania/comments/1r08epg/sticker_shock_85000_pennsylvanians_drop_obamacare/ 26. Understanding the Pharmacy Benefit Reform Act | Insurance Department, <https://www.pa.gov/agencies/insurance/consumer-help-center/learn-about-insurance/health-insu>

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